

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Open-Label, Multicenter, Global Study of Volrustomig (MEDI5752) in Combination With Carboplatin Plus Pemetrexed Versus Platinum Plus Pemetrexed or Nivolumab Plus Ipilimumab in Participants With Unresectable Pleural Mesothelioma (eVOLVE-Meso) **Brief Study Title:** MEDI5752 in Combination With Carboplatin Plus Pemetrexed in Unresectable Pleural Mesothelioma **Short Title/Acronym:** eVOLVE-Meso **Protocol Number:** D7988C00001 **NCT Number:** NCT06097728 **Sponsor Name:** AstraZeneca **Company:** AstraZeneca **Trial Status:** Recruiting (Currently recruiting participants) **Investigational Product:** Volrustomig (MEDI5752) **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Study Information Center / Lead Physician: Dr. Hedy Kindler

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To determine the efficacy and safety of Volrustomig (MEDI5752) + Carboplatin + Pemetrexed vs the investigator's choice of platinum + Pemetrexed or Nivolumab + Ipilimumab in participants with unresectable pleural mesothelioma. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), landmark Overall Survival (OS), landmark PFS, and Objective Response Rate (ORR).

2.2 Background & Rationale Pleural mesothelioma is an aggressive cancer of the lung lining, most commonly caused by asbestos exposure, and is associated with a poor prognosis. Advanced stages cannot be treated with curative surgery, and life expectancy is extremely low even with current standard therapies (chemotherapy or immunotherapy). This study aims to address this critical unmet medical need by evaluating whether the combination of standard chemotherapy with volrustomig (a novel PD-1/CTLA-4 bispecific antibody) can improve survival outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Investigator's choice of Platinum + Pemetrexed OR Nivolumab + Ipilimumab) **Blinding:** Open-label **Randomization:** 1:1, Randomized

3.2 Planned Enrollment & Duration Target \$N\$: 825 participants **Number of Sites:** 178 global locations **Estimated Study Start:** 09-Nov-2023 **Estimated Primary Completion:** 19-Nov-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years at the time of screening. 2. Histologically proven diagnosis of pleural mesothelioma with known histology (epithelioid vs. non-epithelioid). 3. Advanced unresectable disease that cannot be treated with curative surgery (with or without chemotherapy). 4. WHO/ECOG performance status of 0 or 1 with no deterioration over the previous 2 weeks prior to first dosing. 5. Measurable disease per modified RECIST 1.1. 6. Adequate bone marrow reserve and organ function at baseline.

4.2 Exclusion Criteria 1. History of another primary malignancy with exceptions. 2. Untreated or progressive CNS metastatic disease. 3. Active or prior documented autoimmune or inflammatory disorders. 4. Tuberculosis, hepatitis B (HBV) or hepatitis C (HCV), human immunodeficiency virus (HIV) infection that is not well controlled. 5. Any concurrent chemotherapy, radiotherapy, investigational, biologic, or hormonal therapy for cancer treatment. 6. Uncontrolled intercurrent illness. 7. As judged by the investigator, any condition that would interfere with evaluation of the investigational product or interpretation of participant safety or study results.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - *Experimental Arm:* 4 to 6 cycles of Volrustomig + Carboplatin + Pemetrexed in Treatment period 1, followed by Volrustomig alone in Treatment period 2. - *Control Arm:* 4 to 6 cycles of Nivolumab + Ipilimumab or Pemetrexed + Platinum Chemo (Carboplatin or Cisplatin) in Treatment period 1, followed by Nivolumab + Ipilimumab in Treatment period 2. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Up to 2 years (or until disease progression). **Comparator Drug Metadata (Cisplatin):** Drug Name: cisplatin | Preferred Name: cisplatin | Trade Name(s): Platinol | ATC Code: L01XA01

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease symptoms and treatment toxicities. **Prohibited:** Any concurrent systemic chemotherapy, radiotherapy, investigational, biologic, or hormonal therapy for cancer.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Imaging (RECIST 1.1), Safety Labs, ECOG PS
Baseline	Day 0	Randomization, First IV Infusion Dose

| **Interim** | Per Treatment Cycle | IV Infusions, Safety Labs, Efficacy Assessment (Imaging), AE Monitoring | | **End of Study** | Up to 2 Years | End of Treatment Visit, Final Imaging, Follow-up for Survival up to ~61 months |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS) in experimental arm relative to comparator arm - Defined as the time from randomization until the date of death due to any cause.

Measurement Method: Clinical tracking and vital status follow-up evaluated over a timeframe of up to approximately 61 months.

7.2 Secondary & Exploratory Endpoints 1. Progression-Free Survival (PFS) 2. Landmark Overall Survival (OS) 3. Landmark Progression-Free Survival (PFS) 4. Objective Response Rate (ORR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of subjects during each treatment cycle via physical examinations, laboratory testing, and patient interviews. **Serious Adverse Events (SAEs):** Reported within 24 hours of investigator awareness per standard pharmacovigilance protocols. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee appointed to review safety and efficacy data across global sites.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary OS and PFS endpoints. Safety Set for toxicity and adverse event analysis. **Sample Size Justification:** Target enrollment of $N=825$ powered to detect a statistically significant improvement in Overall Survival (OS) between the experimental and standard-of-care arms. **Handling of Missing Data:** Standard event-driven oncology trial censoring rules will apply for OS and PFS endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring of trial centers globally. **Audit Trail:** eCRF entry, central database lock, and rigorous clinical data management audits managed by AstraZeneca.

11. Study Identifiers & Governance

Posting ID: {{854826587529165999}} **Primary ID:** {{D7988C00001}}
Registry Number: {{NCT06097728}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Study Title:** {{MEDI5752 in Combination With Carboplatin Plus Pemetrexed in Unresectable Pleural Mesothelioma}} **Official Regulatory Title:** {{A Phase III, Randomized, Open-Label, Multicenter, Global Study of Volrustomig (MEDI5752) in Combination With Carboplatin Plus Pemetrexed Versus Platinum Plus Pemetrexed or Nivolumab Plus Ipilimumab in Participants With Unresectable Pleural Mesothelioma}}

12. Clinical Framework (Study Specific)

Disease Area: {{Unresectable Pleural Mesothelioma}} **Primary Condition Name:** {{Unresectable Pleural Mesothelioma}} **Preferred Condition Name:** {{Unresectable Pleural Sarcomatoid Mesothelioma}} **Preferred UMLS Name:** {{Malignant pleural mesothelioma, unresectable}} **Condition Definition:** {{Pleural sarcomatoid mesothelioma that is not amenable to surgical resection.}} **Semantic Type:** {{Neoplastic Process}} **MeSH Code:** {{D000085811}} **SNOMED ID:** {{719875003}} **SNOMED Hierarchy:** {{126938006|Malignant mesothelioma|Malignant mesothelioma (disorder)}} **Diagnosis Threshold:** {{Histologically proven diagnosis of pleural mesothelioma (epithelioid vs. non-epithelioid) with advanced unresectable disease}} **Medical Methodology:** {{Standard of Care (Platinum/Pemetrexed or Nivolumab/Ipilimumab) vs Experimental (Volrustomig + Platinum/Pemetrexed)}} **Optimization Target:** {{Overall Survival (OS) and Progression-Free Survival (PFS)}}

13. Study Procedures & Methodology

Management Pathway: {{Intravenous infusion of combination immunotherapy and chemotherapy}} **Education Protocol (HCP):** {{Protocol training, infusion management, and AE identification/reporting}} **Education Protocol (Patient):** {{Informed consent, scheduling education, and toxicity awareness}} **Quality Audit Mechanism:** {{Routine clinical site monitoring and independent data committee review}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 61 months overall, max 2 years on active treatment}} **Onsite Visit Cadence:** {{Prior to each treatment cycle (typically every 3 to 4 weeks depending on assigned arm)}} **Remote Monitoring Frequency:** {{Continuous overall survival follow-up post-treatment}} **Checklist Review Interval:** {{Conducted at every cycle dispensing visit}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					